

Cholera in Nineteenth Century Belfast

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I feel persuaded that epidemic diseases are not the necessary heritage of mankind.

Like other terrestrial phenomena, they have certain causes or precursors. It is frequently a difficult problem to ascertain what these are, although it is a matter of the utmost importance to do so; for without a knowledge of this kind, how can we devise means for arresting the progress or preventing the inroads of epidemics?

Henry McCormac, *Observations on Spasmodic Cholera, Its Origin, Nature and Treatment; With Remarks on Epidemic Diseases Generally* (Belfast: Hodgson, Archer and Jellett, 1832), p. 3.

As Ireland's only industrialised large town Victorian Belfast was, for much of the period, outwardly prosperous and the town displayed a noticeable sense of civic pride which in 1842 led the English novelist William Makepeace Thackeray to remark that Belfast appeared 'hearty thriving and prosperous as if it had money in its pocket and roast beef for dinner.' However, like similar British industrial towns, Belfast had a much less refined underbelly and an urban infrastructure that was largely unfit to cope with a rapidly growing population.

Adjoining many of its main thoroughfares was a network of confined backstreets, entries, courtyards and alleyways, where scenes of deprivation, overcrowding, and poor sanitation could readily be found. Such conditions saw illnesses like fever, dysentery, tuberculosis and many others regularly break out in epidemic proportions; and in February 1832, following months of distressing reports from Europe, Asiatic cholera, a new contagion, unfamiliar in most of world outside areas of the Indian subcontinent where it had originated, made its first appearance in Belfast. Three further cholera epidemics would follow in 1848-49, 1854 and 1866, themselves part of deadly pandemics that swept the globe, all of them influential in altering the world's public health landscape.

Everywhere it struck cholera terrified, and with good reason. While its pandemics approached slowly and could be observed and predicted, limited medical understanding meant that cholera in this period could not be reliably avoided, prevented or cured. It could appear suddenly, spread rapidly and be deadly, infecting rich and poor alike. Its signature symptoms, acute diarrhoea and vomiting, accompanied with agonising stomach and muscle cramps caused catastrophic and rapid dehydration, leaving its comatose corpse-like victims with shrivelled blue-grey skin, and often killing them in just a matter of hours. In some places mortality rates approached those of plague, and in Ireland alone, around 40% of those who contracted cholera between 1832 and 1833 would die as a consequence; in some areas rates were as high as 76%. In 1848/49, mortality rates were even higher, with the disease finding easy prey in the form of a population severely weakened by the impact of the Great Famine and its associated illnesses, and while morbidity rates generally dropped in the outbreaks of 1854-5 and 1866, for most places, Belfast included, mortality rates remained shockingly high.



Image: High Street, Belfast c.1831. Ulster Museum, IC/High St/831.

The Public Health Response in Belfast, 1831-32

Although public health matters were seen as very much the responsibility of local rather than national administrations, in September 1831, with Belfast's Corporation deemed a largely ineffectual body, the first response to cholera in the town was driven by a hastily formed but ambitious Board of Health, comprised of prominent doctors, magistrates and clergymen. The Board, whose formation had ignored the directions of Dublin's General Board of Health (who wanted cholera to be confirmed to their satisfaction prior to the establishment of local boards) was a supervisory body in the first instance, but it quickly assumed responsibility for preparing Belfast for the inevitable arrival of cholera. Beginning by immediately urging those responsible for cleaning the town – the Commissioners and Committee of Police – to step up their efforts, the board also split the town into manageable medical districts with visitors employed to report on the condition of streets and the health of residents. In contrast, other institutions in the town appeared less willing to provide assistance. The Belfast Charitable Society, for example, was canvassed for support but only agreed to supply coffins as required.

A campaign to secure extra hospital accommodation commenced, eventually raising £700 to construct temporary facilities at the rear of the Frederick Street Fever Hospital. A building in Lancaster Street, opposite the hospital, was also acquired to act as a lazaretto in order to house those who had suffered direct contact with cholera victims or were showing symptoms. Initially under the direction of Dr William Duncan, this 'Cholera Hospital' was subsequently transferred to the formidable Dr Henry

McCormac who spearheaded the medical response, employing a strict isolation policy with treatments which included bloodletting and the administration of calomel (mercury), opiates and dilute sulphuric acid. The latter, a method which McCormac claimed was both inexpensive and effective. It seemed, therefore, as Dr Andrew Malcom would recall in 1851, that the early actions of the board meant that 'no town of the same magnitude was placed in more effective defence.'

Cholera's first case in Belfast occurred on 28 February 1832 in a lodging house in Quay Lane, a narrow street near the River Lagan. Bernard Murtaugh, a 34 year old cooper, first showed symptoms around midnight and despite the best efforts of local doctors he died just nineteen hours later. The Board of Health, anxious not to cause undue alarm, was careful not to publicise the death but took strong precautions, burning Murtaugh's bedding, fumigating and whitewashing the house and installing a guard of constables. It was initially suspected that Bernard had contracted the illness from travellers that had arrived from Scotland, and stayed in the same lodgings, but the board later announced that the visitors had been in perfect health. Within days however, several more cases occurred. In Johnny's Court (off Talbot Street), the deaths of George McKeown and his son, 'a stout young man' of 27 along with two others, all of them linked in the *Belfast Newsletter* to the Quay Lane case, forced the board to publicly acknowledge the presence of cholera in the town. Between then and the time of the release of the board's final statistics in November 1832, 2831 cases and 418 deaths were recorded, a mortality rate of just under 15%. While the statistics were shocking, they were still considerably lower than any other large town in Ireland, (Dublin and Cork for example, experienced rates which exceeded 30%). The relative success in managing the crisis was widely praised and attributed principally to the early actions of the Board of Health, its effective cooperation with the town's main sanitary bodies and to the management of the cholera hospital by its medical and ancillary staff.

The Guard Slips

After 1832, new cases were relatively rare, consequently, cholera and on-going preventative public health provision seemed to pass quickly from both civic and public consciousness. Medically, few advances had been made in bacteriological understanding, so when cholera re-emerged in the West after 1847 practically nothing had changed in the way it was fought. However, in Britain and Ireland, public health administration had begun to evolve. Thus, the efforts of the new Board of Guardians, the physician and sanitary reformer Dr Andrew Malcolm and additional sanitary and housing powers, granted to the corporation by town improvement legislation, drove forward the response to Belfast's public health emergencies. The Guardians, for example, acted in defiance of the Poor Law Commissioners when they opened the Belfast Workhouse in 1841 with ten beds in place for the reception of the sick, rapidly increasing this to 100. They also helped the town's medical dispensaries, agreed terms with the General Hospital to accept cholera patients and issued an open letter to local mill and factory owners urging them to release employees who showed cholera

symptoms. (Their actions were justified when thirty-three cases and nine deaths occurred among workers at Ewart's Mill on the Crumlin Road in early February 1849).

The driving force that guided the response of the municipal authorities was undoubtedly Dr Malcolm. During a town meeting in March 1848, he had pointed out numerous public health issues and proposed the formation of the Belfast Sanitary Committee. This body, headed by Malcolm and comprised of prominent community representatives, clergy and local doctors was specifically aimed at dealing with cholera in the first instance. The Committee revived the 1919 Contagious Diseases Act, appointed Officers of Health, and published and distributed reports which gave advice on methods of preventing cholera and better hygienic practice. Magistrates' orders were routinely issued for the removal of nuisances, poor families were provided with straw bedding, houses were whitewashed, and new sewers were constructed in some parts of the town.

Still, some areas where similar sanitary actions were more than warranted remained neglected by the authorities. They included parts of the Smithfield, College and Cromac wards, all of them highlighted in a damning report from the committee in 1849. Each were some of the most notorious seats of cholera, the latter two, the report declared, was due to their proximity to the notoriously unsanitary Blackstaff River, which for some time had been little more than an open sewer that Malcolm called a 'foul and open tortuous stream.' As cholera raged in July 1849, notable deaths in Cromac included the eminent Wesleyan Minister Rev. Matthew Langtree, and in tragic circumstances his wife, who took ill as his remains were being taken for interment. She died shortly after her children returned from the funeral. By mid-October, almost a year after it had commenced the epidemic had run its course. There had been 3,538 cases and 1,163 deaths resulting in a mortality rate (33%) that was more than double that of 1832, yet, Belfast had still managed to avoid the significantly higher tolls recorded in Dublin, Cork and elsewhere across the country.

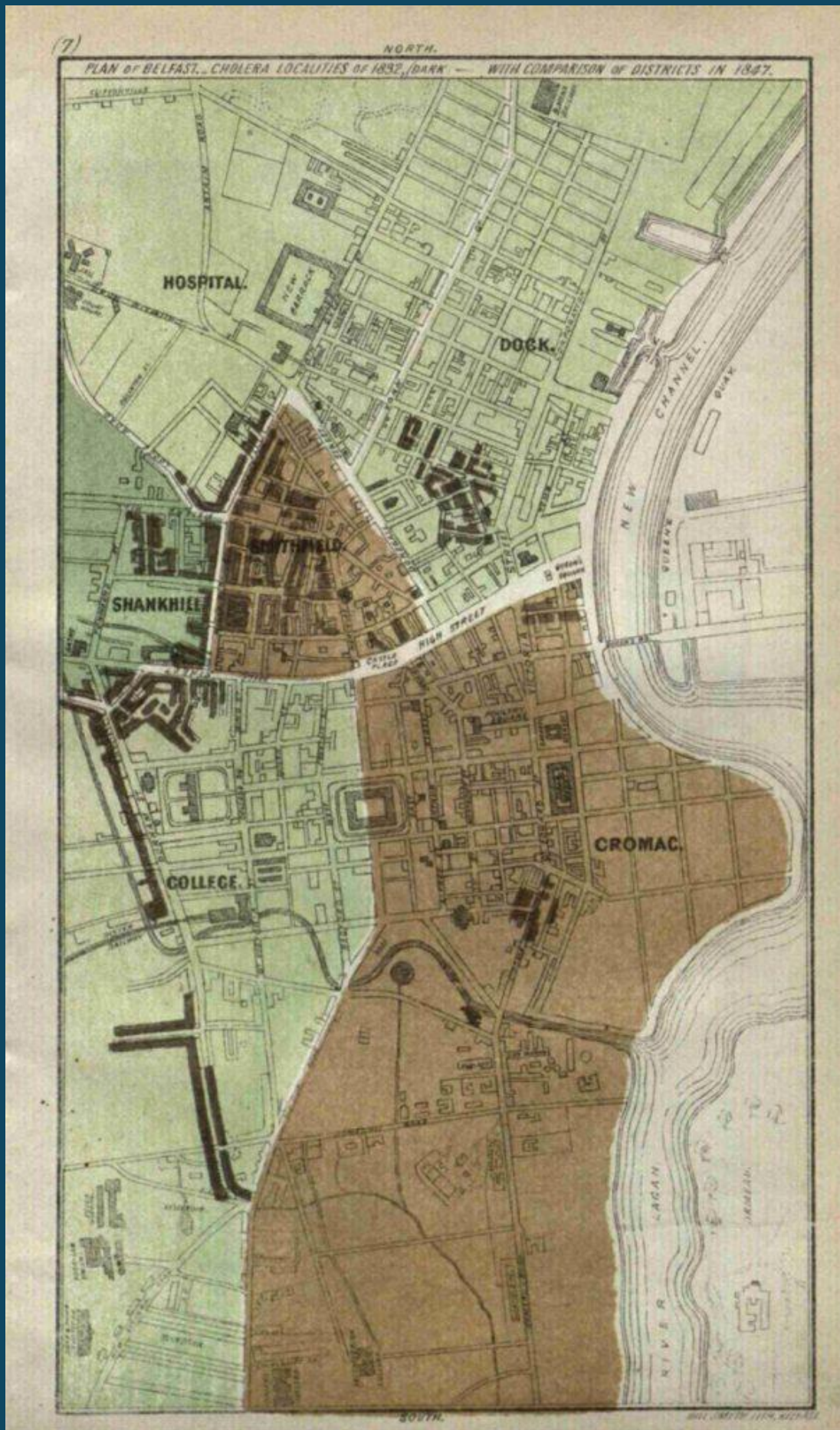


Image: Map of Belfast's Cholera Localities, 1832. Ulster Museum, IC/High St/831.

Were Lessons Learnt?

Following the end of the epidemic, the ensuing decade saw some aspects of health care in Ireland improve. Under the 'Medical Charities (Ireland) Act' (1851) for example, the dispensary system was taken over by the Poor Law Commission, itself restructured to incorporate improved public health administration. Still, complicated sanitary issues would continue to plague Belfast. The Blackstaff nuisance was not addressed and housing and sanitary practices, particularly in the poorest areas of the town had continually been neglected following the end of the previous epidemic. Warning of the threat of another imminent cholera outbreak, Malcolm, speaking to the British Association in 1852, said; 'Are we prepared? We fear not.' But when cholera did return to Belfast in 1854, all the town's municipal bodies made a determined effort to mitigate its impact. Despite this, Belfast was one of the worst affected large Irish towns. The epidemic began in March 1854 when cases were reported in Smithfield and in Washington Street. However, the epidemic's overall duration was relatively short, and it came to an end in October/ November. Accurate statistics are difficult to ascertain, but while case numbers were relatively minimal, fatalities showed a considerable rise. The official returns of Poor Law Commissioners for example recorded 1,871 cases and 677 deaths, (36% mortality). When cholera struck Belfast for the final time in 1866 it could scarcely be considered to have been an epidemic. The official returns recorded just 28 cases of Asiatic cholera with 15 deaths, but the true figure was almost certainly appreciably higher. While this appears to have been a public health success story it really cannot be altogether attributed to any substantial improvement in the town's sanitary condition. Belfast's escape from a major epidemic was therefore less a testament to the effectiveness of its sanitary reform than a consequence of cholera's often unpredictable behaviour, combined with a considerable degree of fortuitous circumstances,

Although occasional alarms were raised, cholera did not return to Ireland in epidemic form after 1866. The decades that followed were dominated by scientific enquiry and efforts aimed towards gaining a better understanding of cholera and other contagious illnesses. This helped significantly change the global medical and public health landscape, and by the end of the century many of the mysteries of epidemic disease had begun to be solved.

Board of Health, BELFAST.

Epidemic Cholera.

IN the end of October, 1831, Cholera appeared in Sunderland. Upon the 17th December, it reached Haddington, in Scotland, and at this date, has appeared both in Glasgow and Paisley. Between these places and Belfast the intercourse is so constant, that we have reason to expect, that within a short period it may reach us, in spite of every precaution. THE BOARD OF HEALTH therefore deem it necessary to address their fellow-townsmen, for the purpose of stating what has been done, what they purpose to do, and what they would advise, with a view either to avert or mitigate the disease.

1. The Town has for several years been divided into six Dispensary Districts, with two Medical Gentlemen to each; but should Cholera appear, there will be an immediate increase of medical appointments, according to the exigency of the case.

2. Each District is placed under the superintendence of Two Members of the Board, each District is again sub-divided, and each Division is placed under Visitors resident in the neighbourhood.

3. The Visitors are engaged in the examination of the Streets, Lanes, Courts, Yards, and Houses; and are endeavouring by

means of the active and zealous co-operation of the Commissioners and Committee of Police to promote the cleanliness of the Town.

4. The Superintendants and Visitors will, wherever necessary, have the houses of the Poor whitewashed, and will furnish a supply of fresh straw for their beds; and be ready to attend to any other matters of precaution or relief, that circumstances may demand.

5. The Quarantine Laws, according to directions received from Government, will be strictly enforced upon all vessels from infected ports.

THE BOARD OF HEALTH beg leave to address to the Public the following suggestions, which, they believe, may, under Divine Providence, contribute to the protection of society in general, and particularly of the poor, who, from a variety of circumstances, may be chiefly exposed to contagion:—

1. Do every thing in your power to maintain cleanliness of person and clothes.

2. To guard against sudden changes of temperature, wear a flannel shirt next the skin, or at least a belt eighteen inches broad round the bowels.

3. As much as possible avoid getting the feet wet, or sitting in wet clothes.

4. Avoid unnecessary and fatiguing journeys.

5. Avoid late hours, and all parties in small close rooms. The ordinary practice of sitting all night at Wakes, is most particularly dangerous.

6. After rising in the morning, let all bed clothes be hung up on a rail or line.

7. Sweep frequently under beds.

8. Keep windows open during the day, whenever the weather is dry. Wherever it can be done, raise one sash, and draw down the other a few inches. Particularly keep open the upper windows of stair-cases, lobbies, &c.

9. Remove from yards, or near the doors, all gatherings of sweepings, dung, &c. causing offensive smells, and drain of all stagnant water.

10. Damp earthen floors being exceedingly prejudicial to health, do all that can be done by drains, or otherwise, to keep them dry.

11. Wash boarded floors frequently, and dry them well, by ventilation and fires.

12. Let SERVANTS and all others keep as much as possible at home, and avoid visiting sick persons, unless where absolute duty requires.

13. As it is not improbable that Dogs, by frequenting infected houses, are often the means of conveying infection to other places, let them be tied up.

14. *Be Temperate.* If Ardent Spirit be used, let it only be in moderate quantity after dinner, and largely diluted with water. Of the tendency of intemperance to produce Cholera, we have a striking example, by which it appears that in consequence of drinking on Christmas-day, (25th December last,) thirty-nine persons were seized with Cholera at Gateshead, upon the 26th; and fifty-nine, upon the 27th: of whom ten died on the 26th, thirty-two on the 27th, and thirteen on the 28th December; whereas the greatest number of deaths upon any other day was only nine, and the ordinary average five. Persons whose business calls them to fairs and markets, are often accustomed to drink spirits after their journey, and in the evening when fatigued with business and weakened by long fasting, a practice at all times injurious to health, and peculiarly calculated to prepare the way for the attack of CHOLERA.

15. A mind at peace is always the best preservative of health. Therefore let all ranks, trusting in the grace of our Lord and Saviour, turn to God with all their hearts. It may be, that, listening to prayer, God will defend us from this scourge; or should he send it amongst us, the visitation will be converted to good.

(Signed by order of the Board,)

Belfast, 18th Feb. 1832.

CHARLES TREVOR,
SECRETARY.

Let this paper be pasted up in Factories, Schools, and Dwelling-houses.